

Uphamol 1000mg Powder (Honey Lemon)



Name and Strength of Active Substance(s)
Paracetamol 1000mg.

PRODUCT DESCRIPTION

Content in sachet: Yellowish colour powder, honey and lemon flavoured
After dissolved in water: Oral solutions appears slightly yellowish in colour and have a lemon odour.

PHARMACODYNAMICS PROPERTIES

Paracetamol is an antipyretic and analgesic. Its mechanism of action is believed to include inhibition of prostaglandin synthesis, primarily within the central nervous system. The lack of peripheral prostaglandin inhibition confers important pharmacological properties such as the maintenance of the protective prostaglandins within the gastrointestinal tract. Paracetamol is, therefore, particularly suitable for patients with a history of disease or on concomitant medication where peripheral prostaglandin inhibition would be undesirable (such as, for example, those with a history of GI bleeding or the elderly).

PHARMACOKINETICS PROPERTIES

Paracetamol is rapidly and almost completely absorbed from the gastro-intestinal tract. It is relatively uniformly distributed throughout most body fluids and exhibits variable protein binding. Paracetamol is metabolized in the liver and excretion is almost exclusively renal, in the form of conjugated metabolites.

INDICATIONS

Paracetamol 1000 mg Powder (Honey and Lemon) can be used to relieve fever, headache and symptoms of flu and cold.

RECOMMENDED DOSAGE

	Dosage
Adult	One sachet, dissolved in a mug or cup of hot water, to be taken every four to six hours as necessary up to a maximum of four sachets in 24 hours.
Children	Not recommended for children under 12 years of age, except on medical advice

ROUTE OF ADMINISTRATION

For oral use.

CONTRAINDICATIONS

Hypersensitivity to paracetamol or any of the other constituents.

WARNING AND PRECAUTIONS

Please consult doctor if symptoms persist. Please do not consume if you are hypersensitive to acetaminophen (paracetamol) or any of the other constituents. Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious conditions. If these reactions occur, stop use and seek medical assistance right away.

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time

INTERACTIONS WITH OTHER MEDICAMENTS

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine. The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Epidemiological studies in human pregnancy have shown no ill effects due to paracetamol used in the recommended dosage, but patients should follow the advice of their doctor regarding its use. Paracetamol is excreted in breast milk but not in a clinically significant amount. Available published data do not contraindicate breast feeding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

None.

ADVERSE EFFECT/UNDESIRABLE EFFECTS

When taken as directed, unwanted effects from Paracetamol 1000mg are rare and usually mild. Haematological reactions including thrombocytopenia, leucopenia, pancytopenia, neutropenia, and agranulocytosis have been reported. Skin rashes, and other hypersensitivity reactions occur occasionally. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

OVERDOSE AND TREATMENT SYMPTOMS

Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported. Immediate treatment is essential in the management of paracetamol

overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the National Poisons Information Service (NPIS) or a liver unit.

STORAGE CONDITIONS

Store below 30°C. Protect from light. Keep out of reach of children.

DOSAGE FORMS AND PACKAGING AVAILABLE

The Paracetamol Powder 1000mg is packed in Laminated Foil Sachet in cartons of 5, 10, 20 and 25 sachets.

NAME AND ADDRESS OF MANUFACTURER/PRODUCT REGISTRATION

PRODUCT REGISTRATION HOLDER

DUOPHARMA MARKETING SDN. BHD.
Lot No. 2,4,6,8 & 10, Jalan P/7, Section 13, Bangi Industrial Estate,
43650 Bandar Baru Bangi, Selangor, Malaysia.

MANUFACTURER

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.
Lot No. 2 & 4, Jalan P/7, Section 13, Bangi Industrial Estate,
43650 Bandar Baru Bangi, Selangor, Malaysia.

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Size: 23.2cm(H) x 15cm(W)

Black 100%